

CJC-1295 (No DAC)

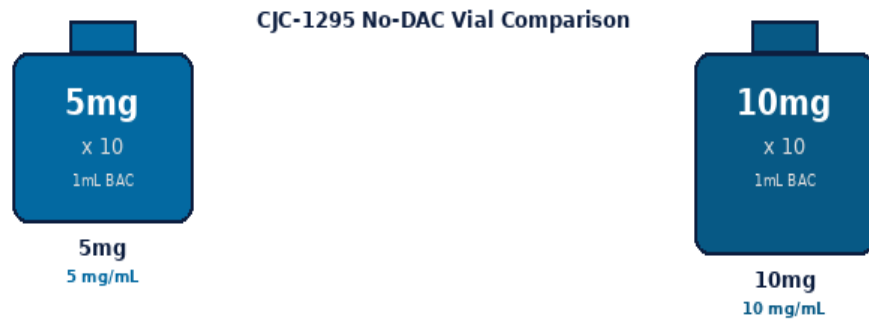
GHRH Analogue | Short-Acting | 5mg & 10mg x 10 Vials

Inject 2-3x Daily Fasted | Stack with Ipamorelin for Amplified GH

Purity: 99.8% | Endotoxin-Free | Research Use Only

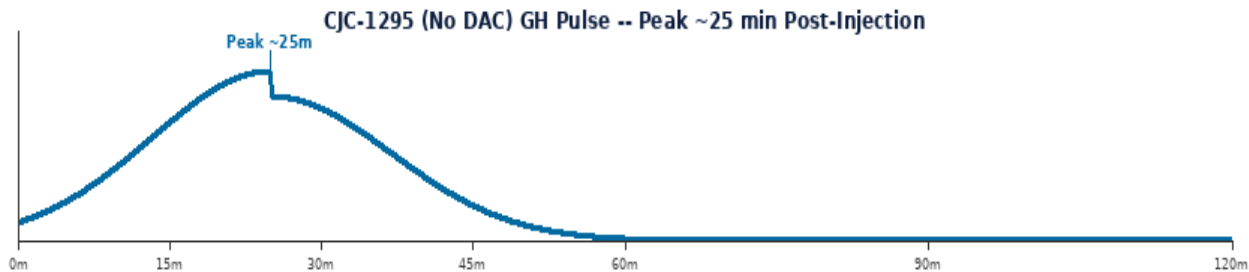
CJC-1295 without DAC (Modified GRF 1-29) is a short-acting GHRH analogue (half-life ~30 min) that stimulates pituitary GH release. Must be injected fasted. Stacked with Ipamorelin or GHRP-2 at same injection for 3-10x amplified GH pulses.

FASTED REQUIRED: Inject at least 1-2 hours after last meal. Insulin blunts GH release. Wait 30 min before eating post-injection.



Parameter	Specification
Half-Life	~30 min (without DAC) -- inject 2-3x daily fasted
Vial Sizes	5mg x 10 10mg x 10 1 mL BAC Water Purity 99.8%
Concentration	5mg -> 5 mg/mL (5,000 mcg/mL) 10mg -> 10 mg/mL (10,000 mcg/mL)
Standard Dose	100-300 mcg per injection start at 100 mcg
Frequency	2-3x daily -- ALWAYS FASTED (1-2hr after last meal)
Best Timing	Morning fasted + pre-bed -- aligns with natural GH pulse windows
Stack With	Ipamorelin or GHRP-2/6 -- inject simultaneously -- amplified GH 3-10x
Cycle & Storage	12 weeks on, 4 weeks off 2-8 deg C reconstituted use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Research Benefit
GHRHR Activation	Binds GHRH receptors on pituitary somatotrophs; activates Gs/cAMP/PKA cascade; stimulates GH gene expression and granule exocytosis; half-life ~30 min mimics natural GHRH pulse	Clean physiological GH pulse; no cortisol/prolactin
GHS Synergy	GHRH primes pituitary somatotroph stores and enlarges GH pool; GHS (Ipamorelin) triggers release via separate GHSR-1a pathway; combined = 3-10x GH pulse vs either alone	Maximum GH output when stacked; optimal body composition protocol
IGF-1 Elevation	Repeated GH pulses stimulate hepatic IGF-1 production; rises progressively over weeks; mediates anabolic, lipolytic, and recovery effects	Lean mass, fat loss, recovery, sleep quality

Benefit	Research Context
Physiological GH Pulse	Short half-life mimics natural GHRH precisely, producing clean GH release without desensitisation when cycled correctly.
GHS Synergy	Stacked with Ipamorelin or GHRP-2/6 at the same injection produces amplified GH pulses 3-10x greater than either peptide alone.
No Fasting Concerns vs DAC	2-3x daily injections allow flexible GH pulse control aligned with natural secretion patterns throughout the day.

RECONSTITUTION & DOSE CALCULATION

Both vials use 1 mL BAC Water. 5mg->5mg/mL | 10mg->10mg/mL. Room temperature. Swirl gently.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



1mL Insulin Syringe | 29-31G | SubQ

Formula: Vol(mL)=Dose(mcg)/Conc(mcg/mL) | **Units:** Vol x 100

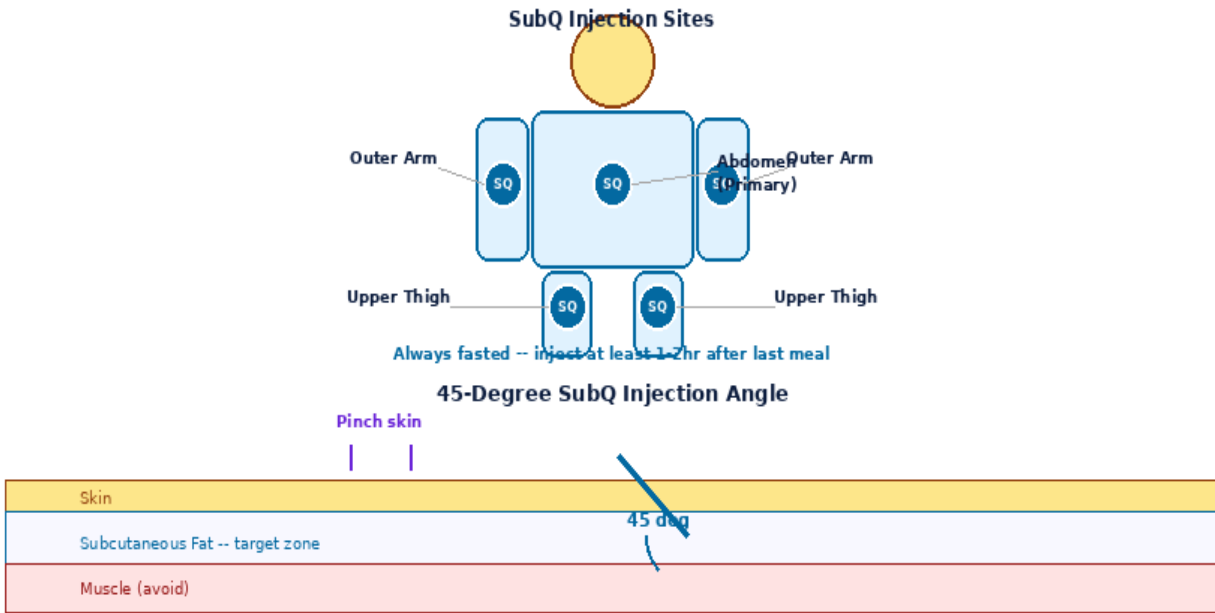
5mg x 10 -- 1mL BAC -> 5.0mg/mL (5,000mcg/mL)

Dose(mcg)	100mcg	150mcg	200mcg	250mcg	300mcg
Vol(mL)	0.0200	0.0300	0.0400	0.0500	0.0600
Units	2.0U	3.0U	4.0U	5.0U	6.0U
Doses/Vial	50	33	25	20	16

10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	100mcg	150mcg	200mcg	250mcg	300mcg
Vol(mL)	0.0100	0.0150	0.0200	0.0250	0.0300
Units	1.0U	1.5U	2.0U	2.5U	3.0U
Doses/Vial	100	66	50	40	33

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Confirm fasted	1-2hr after last meal. Mandatory.
2	Draw dose	29-31G syringe. Exact volume. If stacking, CJC+Ipamorelin can share one syringe.
3	Inject SubQ	Pinch 2-3cm. 45 deg. Slowly. Rotate 8-point map.
4	Wait & dispose	Wait 30 min before eating. Sharps container.

Protocol	Dose	Frequency	Duration	Notes
Beginner	100 mcg	2x daily bed+AM	12wk/4off	Start here. Both fasted.
Standard	200 mcg	2-3x daily fasted	12wk/4off	Common protocol.
Stack (Best)	200mcg CJC +200mcg Ipa	2x daily fasted	12wk/4off	Combine in syringe. 3-10x GH.

Side Effect	Likelihood	Management
Injection site reaction	Common	Rotate sites. Fresh needle.
Water retention	Occasional	GH-mediated. Reduce dose if significant.
Headache / tingling	Occasional	First 1-2 weeks. Resolves.

Contraindications: Cancer history, pregnancy, under 18.

1. Always inject fasted (1-2hr after last meal). Insulin kills the GH pulse.
2. Both vials use 1 mL BAC Water: 5mg->5mg/mL 10mg->10mg/mL.
3. Stack with Ipamorelin at same injection for 3-10x amplified GH pulse.
4. CJC-1295 and Ipamorelin are compatible -- draw into one syringe.
5. Inject 2-3x daily timed to natural GH windows: morning fasted + pre-bed.
6. Wait 30 minutes post-injection before any caloric intake.
7. Rotate through 8-point abdomen map. Swirl -- never shake.
8. Cycle 12 weeks on, 4 weeks off. Store 2-8 deg C. Use within 28 days.
9. Contraindicated in cancer history. Not for pregnancy or under 18.

DISCLAIMER:

research use only. Not for human therapeutic use. Consult a qualified professional.

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